

DESIGN ADDENDUM

Submission & Receipt Verification

Surgery Authorization AI App — September 3, 2026

Purpose of This Addendum

This addendum adds a submission-control layer to the authorization workflow. A clinically complete, policy-matched letter is not enough if a payer's reviewer later states that the MRI, physical-therapy records, pain-management notes, imaging addendum, or physician letter was never received or was not available to the reviewer. The app should therefore track not only what was written, but exactly what was transmitted, when it was transmitted, how it was transmitted, whether the payer disputed receipt, and what the reviewer later claimed was available.

1. New Core Principle: Evidence Chain of Custody

Every document should move through explicit states:

- Uploaded from chart
- Validated as the correct/current document
- Mapped to one or more policy criteria
- Selected for payer submission
- Transmitted
- Transmission evidence captured
- Receipt verification requested
- Carrier explicitly confirmed receipt OR carrier did not dispute the documented submission
- Availability to physician reviewer confirmed, when obtainable
- Reviewer-file status checked before peer-to-peer

The GUI must preserve the distinction between transmission and receipt. Silence from a carrier should never be labeled as an affirmative legal confirmation of receipt unless an applicable rule actually creates that presumption.

2. Required Workflow Extension

Surgeon approves final letter + attachment package



App creates document-by-document submission manifest



Package transmitted through verified payer channel



App captures fax confirmation / portal receipt / secure-email evidence



Automated receipt-verification message sent through appropriate channel



Carrier confirms? → YES: mark Confirmed Received



Carrier does not respond? → mark Transmission Documented — Carrier Did Not Dispute



Carrier identifies missing/incomplete item? → immediate staff task to resend



Before peer-to-peer: reviewer-file verification audit



Reviewer claims missing record? → capture discrepancy contemporaneously



Material discrepancy + adverse determination? → regulatory/escalation workflow

3. Submission Manifest

Every outbound authorization or appeal package should generate a manifest that becomes part of the permanent case record. The manifest should identify each attachment by a human-readable name, document date, source/provider, page count when available, and its authorization importance.

Suggested evidence classifications:

- CRITICAL — required to satisfy a selected payer-policy criterion.
- SUPPORTING — materially strengthens medical necessity but is not independently required.
- BACKGROUND — relevant context but not necessary to establish the chosen pathway.

4. Automated Receipt-Verification Message

The app should generate a receipt-verification message after each material submission. It should not merely ask 'Did you get our records?' It should force the communication to identify the exact submission being referenced.

Subject: Confirmation of Records Submitted — [Patient] — [Case/Auth #]

On [DATE], our office transmitted the records listed below in support of the above authorization/appeal:

[DOCUMENT MANIFEST]

Please notify our office promptly if any listed item was not received, is incomplete or illegible, or is not available to the physician who will conduct the utilization review. If we receive no notice identifying a missing or unavailable item, our office will rely upon this transmission record in proceeding with the authorization and peer-review process.

If a subsequent adverse determination or peer-review discussion asserts that a listed record was not available for review, we will compare that assertion with our transmission documentation and this notice. Material discrepancies in the handling or availability of submitted clinical information may be referred, when appropriate, to the governmental or regulatory entity having jurisdiction over the health plan or utilization-review process.

Thank you,
Office of Kevin B. James, MD

5. Why Silence Must Be Represented Carefully

Some carriers — particularly workers' compensation carriers — may not answer a request to verify every item received. The app should anticipate this rather than treating nonresponse as workflow failure. However, the office should not unilaterally state that silence legally equals receipt unless the governing law, rule, contract, or payer procedure supports that conclusion.

Instead, the verification notice creates a contemporaneous record: the office identifies exactly what it transmitted, provides the carrier an opportunity to identify missing or unusable material, and states that the office will rely on its documented transmission record if no discrepancy is reported.

Recommended status taxonomy:

- CONFIRMED RECEIVED — payer affirmatively confirms the item.
- TRANSMISSION DOCUMENTED — CARRIER DID NOT DISPUTE — valid transmission evidence exists and the carrier did not identify the item as missing after verification request.
- RECEIPT DISPUTED / ITEM MISSING — payer says the item is absent, incomplete, illegible, or unavailable.
- AVAILABLE TO REVIEWER CONFIRMED — payer/reviewer specifically confirms access to the item.
- REVIEWER AVAILABILITY UNKNOWN — transmitted/received status exists but reviewer access has not been confirmed.

6. Payer Communication Profiles

Do not hard-code one universal email address. The app should maintain payer- and jurisdiction-specific communication profiles. A correspondence address that works for one appeal may not be the payer's official medical-record submission channel.

- Official authorization/appeal submission channel
- Provider portal and/or utilization-management vendor
- Official appeal email/fax
- Case-specific secure correspondence channel
- Receipt-verification channel
- PHI/security requirements
- Expected acknowledgement behavior
- Source used to verify contact information and verification date
- State/federal jurisdiction and applicable regulator

The BCBS Massachusetts Albers case is the model for this distinction: the patient's secure correspondence with askappeals@bcbsma.com demonstrated a functioning Member Appeals and Grievances inquiry channel and produced an explicit statement identifying records that had been reviewed. The app should distinguish such a case-correspondence channel from the independently verified official channel used to submit the medical record.

7. Peer-to-Peer Preflight

Twenty-four to forty-eight hours before a scheduled peer-to-peer, the app should generate a reviewer-file preflight for staff and the surgeon.

- Selected payer policy and version
- Chosen authorization pathway
- Policy-critical evidence list
- Transmission date/method for each critical item
- Receipt status for each critical item
- Any carrier-disputed or unconfirmed items
- Latest imaging and all addenda
- PT documentation required by the selected pathway
- Pain-management records required by the selected pathway
- Current surgeon letter
- Denial letter and stated denial rationale

Example display:

MRI 08/11/26 — Sent ✓ | Carrier confirmed ✓ | Reviewer availability ✓
 MRI addendum — Sent ✓ | Carrier did not dispute | Reviewer availability unknown
 PT 04/02–06/30 — Sent ✓ | Carrier confirmed ✓
 Dr. James letter — Sent ✓ | Carrier confirmed ✓
 Missing policy-critical items — NONE

8. Capture the Peer-to-Peer Discrepancy

If the reviewing physician says a record is missing, the app should make that event easy to document immediately rather than reconstruct it weeks later.

- Date and time
- Reviewer name
- Reviewer specialty/subspecialty if known
- Exact document the reviewer states is unavailable
- Reviewer statement or close contemporaneous paraphrase
- Original transmission date and method
- Transmission confirmation identifier
- Receipt-verification request date
- Carrier response or nonresponse
- Whether staff resent the document during/after the call
- Whether the missing record was material to the denial

9. Regulatory Escalation Logic

A later assertion that policy-critical evidence was absent should be compared automatically with the submission chain of custody. A discrepancy should not automatically generate an accusation or complaint. It should generate a reviewable event.

If the discrepancy is material — for example, an adverse determination says PT was not provided despite documented transmission and a prior verification notice — the app should identify the governmental or regulatory body with jurisdiction and prepare the factual record for possible escalation.

- Determine plan type and jurisdiction first; insurer brand alone is insufficient.
- Distinguish a medical-necessity appeal from a complaint about utilization-review process.
- For workers' compensation, determine the specific state's workers' compensation regulator/utilization-review process.
- For state-regulated commercial insurance, determine the state insurance/external-review authority.
- For applicable self-funded ERISA plans, determine the relevant federal route.
- Use professional language: request review of process, reviewer qualifications, handling of submitted evidence, or application of incorrect criteria — not punishment for a good-faith disagreement.

10. New GUI Components

- Submission Package screen — final letter plus attachment manifest.
- Transmission screen — channel, destination, date/time, sender, confirmation number/file.
- Receipt Verification screen — message sent, response deadline/follow-up, payer response.
- Document Status grid — Sent / Confirmed / Not Disputed / Missing / Reviewer Access Unknown.
- Peer-to-Peer Preflight screen — policy-critical evidence and reviewer-file readiness.
- Peer-to-Peer Call Log — reviewer identity, missing-document claims, discussion outcome.
- Discrepancy/Escalation screen — compares denial/reviewer assertions against transmission evidence.
- Regulatory Route panel — governing body, verified contact path, basis for jurisdiction, and complaint-ready factual chronology.

11. New Developer Data Objects

- SubmissionManifest: case ID, submission date, purpose, document IDs, criticality, page counts.
- TransmissionEvent: channel, destination, timestamp, sender, confirmation ID, evidence artifact.
- ReceiptVerification: manifest ID, request timestamp, destination, response status, payer response.
- DocumentReceiptState: document ID, transmitted, confirmed received, not disputed, disputed/missing, reviewer-access status.
- PeerReviewEvent: date/time, reviewer identity/specialty, documents claimed missing, outcome, contemporaneous notes.
- EvidenceDiscrepancy: payer/reviewer assertion, conflicting transmission evidence, materiality, resolution.
- PayerCommunicationProfile: verified submission routes, correspondence routes, PHI rules, source, last-verified date.
- RegulatoryJurisdiction: plan type, state/federal/WC authority, regulator, external-review route, confidence/source.

12. Updated Quality-Control Rules

- No package is marked READY until all CRITICAL evidence has been selected and transmitted.
- Transmission evidence must be stored for every outbound package.
- A receipt-verification notice should follow each material submission when an appropriate communication route exists.
- Carrier silence must not be mislabeled as affirmative confirmation.
- The app should flag any critical item whose reviewer availability remains unknown before peer-to-peer.

- Any reviewer claim of a missing critical item must be compared against the transmission record.
- The app should preserve all correspondence and confirmations for appeal/external review.
- Regulatory language must be jurisdiction-specific and professional.
- The final authorization letter remains dated and includes the surgeon's approved electronic signature/signature block.

13. Product Design Outcome

The original design makes the AI solve the medical-policy problem before writing. This addendum makes the application solve the evidence-delivery problem after writing. Together, the system should be able to answer two questions at any point in the authorization lifecycle:

1. Why does this patient satisfy this exact payer pathway?

2. Can we prove that the payer had the evidence needed to evaluate it?

That second question is particularly important before peer-to-peer review. The goal is to prevent a clinically supportable authorization from failing because a policy-critical document was allegedly unavailable, and to create a clean factual record when the payer's handling of submitted evidence becomes part of the appeal or regulatory issue.